

EXPLANATORY STATEMENT

My Health Records Act 2012

My Health Record (Share by Default) Rules 2025

Purpose and operation

The *My Health Record (Share by Default) Rules 2025* (the Instrument) sets out the scope of healthcare services and providers subject to obligations to upload information to the My Health Record system by default.

The Instrument gives effect to amendments to the *My Health Records Act 2012* (the Act) made by the *Health Legislation Amendment (Modernising My Health Record - Sharing by Default) Act 2025* (SBD Act). These reforms set out a new regulatory framework for mandating the sharing of specified health information to the My Health Record system. The Instrument specifies proprietors of pathology labs and diagnostic imaging premises as constitutional corporations who will be required to become registered under the Act and upload prescribed information. The Instrument outlines the information required to be uploaded are pathologist and radiologist reports resulting from specified pathology and diagnostic imaging services. Uploading to My Health Record will be required within 24 hours of the report being sent to a requesting or treating practitioner. Reports will be required to be uploaded regardless of whether medicare benefits are payable for those services. The upload requirement is subject to exceptions outlined in section 10B of the Act, including when:

- a. the healthcare recipient or their representative has advised that they do not want the information uploaded
- b. a healthcare provider reasonably believes that the information should not be uploaded due to a serious concern for the health, safety or wellbeing of the healthcare recipient
- c. the healthcare recipient is not registered for My Health Record
- d. the information cannot be shared due to other reasons beyond the control of the healthcare provider.

Background

Sharing of information to an individual's My Health Record is currently voluntary for healthcare providers. The SBD Act, passed in February 2025, amended the Act and the *Health Insurance Act 1973* (the HI Act) to implement a new framework to mandate the sharing of individuals' health information to My Health Record for the first time. The SBD Act reforms responded to recommendations in the Strengthening Medicare Taskforce Report released in 2022. The report highlighted that access to real time health information is a critical foundation for a modern and connected health care system and recommended modernising My Health Record to significantly increase health information available to consumers and their healthcare providers by:

- requiring 'sharing by default' for private and public practitioners and services, and
- making it easier for people and their health care teams to use at the point-of-care.

The SBD Act amended the Act to create an obligation on specified constitutional corporations to become registered under the Act and to upload specified information. The HI Act was also amended to make medicare benefits for prescribed services only payable when prescribed information has been uploaded. The amendments in the SBD Act ensure that where an obligation applies to either a constitutional corporation or an individual healthcare provider or both, the obligation will be satisfied if either uploads the prescribed information. The initial scope of the proposed mandate is reports authorised by pathologists and radiologists as outlined in this Instrument as well as rules made under the HI Act.

Authority

Section 109 of the Act provides that the Minister may make rules about matters required or permitted by this Act to be dealt with in the My Health Records Rules.

Section 78A provides that rules may specify the types of healthcare that the upload obligation applies to, the type of information that must be uploaded, and the period within which it must be uploaded. Paragraph (b) of the definition of ‘prescribed healthcare provider organisation’ in section 5 of the Act provides that rules may specify the constitutional corporations which must meet the obligation in section 78A of the Act to upload certain information to My Health Record.

Section 70AA of the Act provides that the rules may specify health information contained in My Health Record to be collected, used and disclosed for compliance purposes.

Commencement

This instrument commences on 1 July 2026.

Consultation

The SBD Act and this Instrument have been developed following extensive consultation with the relevant industry sectors, medical peak bodies, consumer bodies, other Australian jurisdictions and the Australian public.

There was broad support from Australian governments and stakeholders for sharing pathology and diagnostic imaging reports by default to My Health Record.

A Clinical Reference Group was established by the Australian Digital Health Agency to provide clinical and strategic advice to support safe implementation of these reforms, including proposed exceptions and safeguards. The clinical and strategic advice and recommendations from the Clinical Reference Group will also be incorporated in the development of a Clinical Safety Guide to assist in implementing these reforms. This group has been replaced by an ongoing National Clinical Governance Committee for Digital Health. The Committee will evaluate the implementation and effectiveness of these reforms as well as providing ongoing clinical guidance.

General

This instrument is a legislative instrument for the purposes of the *Legislation Act 2003*.

Details of this instrument are set out in **Attachment A**.

This instrument is compatible with the human rights and freedoms recognised or declared under section 3 of the *Human Rights (Parliamentary Scrutiny) Act 2011*. A full statement of compatibility is set out in **Attachment B**.

Details of the *My Health Record (Share by Default) Rules 2025*

Section 1 – Name

Section 1 provides that the name of the instrument is the *My Health Record (Share by Default) Rules 2025*.

Section 2 – Commencement

This instrument commences on 1 July 2026.

Section 3 – Authority

Section 3 provides that the instrument is made under the *My Health Records Act 2012 (the Act)*.

Section 4 – Definitions

Section 4 provides the following definitions for this instrument:

Act is defined as the *My Health Records Act 2012*

diagnostic imaging premises has the same meaning as in the Health Insurance Act

diagnostic imaging service has the same meaning as in the Health Insurance Act

Health Insurance Act is defined as the *Health Insurance Act 1973*

medicare benefit is defined as having the same meaning as in the Health Insurance Act

pathologist is defined as a medical practitioner that is:

- a. recognised as a specialist in pathology under subsection 3D(1) of the Health Insurance Act; or
- b. recognised as a specialist in pathology by a determination under paragraph 3DB(4)(a) or subsection 3E(1) of the Health Insurance Act that is in effect; or
- c. who is an approved pathology practitioner (within the meaning of the Health Insurance Act)

pathology service is defined as having the same meaning as in the Health Insurance Act

proprietor, in relation to a pathology laboratory or diagnostic imaging premises, is defined as having the same meaning as in the Health Insurance Act

radiologist is defined as a medical practitioner who is recognised as a specialist in diagnostic radiology:

- a. under subsection 3D(1) of the Health Insurance Act; or

- b. by a determination under paragraph 3DB(4)(a) or subsection 3E(1) of the Health Insurance Act that is in effect.

R-type diagnostic imaging service is defined as having the same meaning as in the Health Insurance Act.

The note to section 4 also clarifies that a number of terms used in this instrument are defined in the Act including healthcare, healthcare provider organisation and My Health Record system.

Part 2 – Sharing health information by default

Division 1 – Diagnostic imaging services

Section 5 – Diagnostic imaging services—prescribed healthcare provider organisations

Section 5 provides that a proprietor of diagnostic imaging premises is a prescribed healthcare provider organisation within the meaning of paragraph (b) of the definition in section 5 of the Act. The terms ‘proprietor’ and ‘diagnostic imaging premises’ take the same meaning as in the HI Act as provided in section 4.

Under the definition in section 4, proprietors of diagnostic imaging premises that are constitutional corporations will be prescribed healthcare provider organisations. Prescribed healthcare provider organisations are required to be registered for My Health Record as provided in section 41A, upload information as required in section 78A and keep relevant records and display notices when not sharing information to My Health Record as required in sections 78C and 78D.

Section 5 clarifies that a proprietor of diagnostic imaging premises will only be a prescribed healthcare provider organisation if they provide the services specified in section 6.

Section 6 – kinds of healthcare

Section 6 specifies the types of diagnostic imaging services that are healthcare for the purposes of paragraph 78A(1)(b) of the Act. These kinds of healthcare are impacted by the requirement to upload to My Health Record, subject to any additional exceptions outlined in the Act or rules. The following kinds of healthcare are specified:

- a. R-type services listed in tables provided under Part 2 – Services and fees of the Health Insurance (Diagnostic Imaging Services Table) Regulations (No. 2) 2020 or additionally determined by the Minister to be treated as if it were in that table
- b. diagnostic procedures and investigations listed in part 4 of the Health Insurance (General Medical Services Table) Regulations (No. 2) 2021 or additionally determined by the Minister to be treated as if it were in that table
- c. any other service rendered by or on behalf of a radiologist for which a report is generated. This includes services that are not covered in any legislative instrument made under the HI Act, and for which Medicare benefits are not payable.

For the purposes of this section, these services must be provided by or on behalf of a radiologist to be in scope of the upload requirement. The upload requirement will apply whether or not Medicare benefits are payable for the service.

Subsection 6(2) specifies certain types of tests that are exempt from the upload requirement. Subsection 6(1) does not apply to the following kinds of healthcare, which are tests that are usually done for purposes unrelated to a person's health and comprise:

- a. workplace drug or alcohol testing
- b. court ordered testing
- c. testing done for law enforcement purposes

Reports from these types of services can still be uploaded but are not required to be.

In relation to workplace drug and alcohol testing, as noted, these tests are excepted as they are generally preformed for reasons unrelated to the provision of healthcare. As other diagnostic tests performed for work-related purposes may have clinical relevance for an individual, they are subject to the upload requirements. The exceptions in this section operate alongside the exceptions in the SBD Act noted above, including that information does not need to be shared where an individual healthcare provider reasonably believes that the information should not be shared because of a serious concern for the health, safety or wellbeing of the healthcare recipient. Discretion not to upload in such cases would be on a case-by-case basis, having regard to the individual circumstances for the healthcare recipient. This exception may be triggered by a requesting healthcare provider, who indicates that the results of the diagnostic service should not be shared. A radiologist would also be expected to follow existing critical results management processes, including giving consideration as to whether to exercise discretion to rely on the health, safety and wellbeing exception, following applicable clinical guidelines in this regard.

Section 7 - Diagnostic imaging services—information to be shared with the My Health Record system

Section 7 specifies the information that needs to be uploaded to the My Health Record system. It specifies for the purposes of subsection 78A(1), the information contained in a report or other document that is created in relation to the healthcare outlined in section 6 that is authorised by the radiologist who provided the healthcare, or on whose behalf the healthcare was provided. Subsection 7(2) specifies that images are excluded. Only information from written reports is required to be uploaded.

Report is the generally accepted term to reflect the format of information that is provided to a requesting or treating healthcare practitioner, or to the individual themselves, outlining the results of a particular diagnostic service. The effect of these rules is that the information or data elements contained in that report, or other document, are to be shared to the individual's My Health Record, in accordance with the system specifications developed by the My Health Record System Operator.

Section 8 - Diagnostic imaging services—period for sharing information with the My Health Record system

Section 8 specifies the time period within which the information specified in section 7 must be uploaded to My Health Record. Information needs to be uploaded within 24 hours of when the information is first provided to any of the following:

- a. the healthcare provider who requested the healthcare to which the information relates;
- b. the healthcare provider who is treating the healthcare recipient to whom the information relates;
- c. the healthcare recipient to whom the information relates.

This timeframe supports technical implementation matters, such as where reports may be sent to My Health Record in batches, or scheduled for upload outside of normal business hours. It reflects expectations that information should be shared to My Health Record at the same time as it is shared to the healthcare recipient's requesting or treating healthcare provider, or to the recipient where they directly requested the service.

Section 9 - Diagnostic imaging services—kinds of health information that may be collected, used or disclosed

Section 9 specifies the types of health information contained in the My Health Record System that can be collected, used or disclosed for the purposes of enforcing the requirements of a share by default provision. Section 5 of the Act sets out the provisions that are share by default provisions. For the purposes of section 70AA this information is limited to:

- a. the type of a report or other document (i.e. diagnostic imaging report - it will not specify what type of test or scan)
- b. the time at which, and the date on which, a diagnostic imaging service is requested or rendered.

This information is able to be disclosed to the Chief Executive Medicare, the Secretary of the Department and any other Commonwealth entity specified in the rules. This information is prescribed in the Instrument rather than the Act to ensure it can be limited to only what is necessary for compliance purposes, which may be different depending on the kinds of healthcare and information being specified. If different healthcare and information are specified for the purposes of section 78A(1) in the future, then different information may be required for compliance purposes under section 70AA, which would be subject to further consultation prior to prescribing the additional information.

Under subsection 70AA(4), healthcare recipient only notes are not able to be collected, used or disclosed for compliance purposes.

Division 2 – Pathology services

Section 10 – Pathology services—prescribed healthcare provider organisations

Section 10 provides that a proprietor of a pathology laboratory is a prescribed healthcare provider organisation within the meaning of paragraph (b) of the definition in section 5 of the Act. The term 'proprietor' takes the same meaning as in the HI Act as provided in section 4, which in relation to a pathology laboratory, is a person or authority that has effective control of:

- a. the laboratory premises, whether or not the holder of an estate or interest in the premises; and
- b. the use of equipment used in the laboratory; and
- c. the employment of staff in the laboratory.

Pursuant to the definition in section 4, proprietors of pathology laboratories that are constitutional corporations will be prescribed healthcare provider organisations. Prescribed healthcare provider organisations are required to be registered for My Health Record as provided in section 41A, upload information as required in section 78A and keep relevant records and display notices when not sharing information to My Health Record as required in sections 78C and 78D.

Section 10 clarifies that a proprietor of pathology laboratories will only be a prescribed healthcare provider organisation if they provide the healthcare services specified in section 11.

Section 11 – kinds of healthcare

Section 11 specifies the types of pathology services that are healthcare for purposes of paragraph 78A(1)(b) of the Act. These kinds of healthcare are impacted by the requirement to upload to My Health Record, subject to any additional exceptions outlined in the Act or rules. The following kinds of healthcare are specified:

- a. a pathology service referred to in an item of the table in Part 2 of Schedule 1 to the Health Insurance (Pathology Services Table) Regulations 2020 or additionally determined by the Minister to be treated as if it were in that table; and
- b. any other kind of healthcare rendered by or on behalf of a pathologist for which a report is created.

For the purposes of this section, these services must be provided by or on behalf of a pathologist to be in scope of the upload requirement. The upload requirement will apply whether or not medicare benefits are payable for the service.

Subsection 11(2) specifies certain types of tests that are exempt from the upload requirement. Subsection 11(1) does not apply to the following kinds of healthcare, which are tests that are usually done for purposes unrelated to a person's health and comprise:

- a. workplace drug or alcohol testing
- b. court ordered testing
- c. testing done for law enforcement purposes

Subsection 11(3) specifies specific treatment for pathology services conducted for the sole purpose of a research study or clinical trial. For these kinds of healthcare, information specified in section 12 for upload to the My Health Record system only needs to be uploaded if it is otherwise provided to the healthcare recipient and/or their treating healthcare provider. Where the information is so provided, the upload requirement will operate so that the information must also be shared to the individual's My Health Record within the specified timeframe of 24 hours of providing to the individual or their treating provider, as provided by sections 12 and 13 of these rules.

Information relating to the kinds of healthcare specified in subsections 11(2) and (3) can still be uploaded even where it is not required to be.

In relation to workplace drug and alcohol testing, as noted, these tests are excepted as they are generally preformed for reasons unrelated to the provision of healthcare. As other tests performed for work-related purposes may have clinical relevance for an individual, they are subject to the upload requirements. The exceptions in this section operate alongside the exceptions in the SBD Act noted above, including that information does not need to be shared where an individual healthcare provider reasonably believes that the information should not be shared because of a serious concern for the health, safety or wellbeing of the healthcare recipient. Discretion not to upload in such cases would be on a case-by-case basis, having regard to the individual circumstances for the healthcare recipient. This exception may be triggered by a requesting healthcare provider, who indicates that the results of the pathology

service should not be shared. A pathologist would also be expected to follow existing critical results management processes, including giving consideration as to whether to exercise discretion to rely on the health, safety and wellbeing exception, following applicable clinical guidelines in this regard.

Section 12 - Pathology services—information to be shared with the My Health Record system

Section 12 specifies the information that needs to be uploaded to the My Health Record system. It specifies for the purposes of subsection 78A(1), the information contained in a report or other document that is created in relation to the healthcare outlined in section 11 that is authorised by the pathologist who provided the healthcare, or on whose behalf the healthcare was provided.

Report is the generally accepted term to reflect the format of information that is provided to a requesting or treating healthcare practitioner, or to the healthcare recipient themselves, outlining the results of a particular pathology service. The effect of these rules is that the information or data elements contained in that report, or other document, are to be shared to the individual's My Health Record, in accordance with the system specifications developed by the My Health Record System Operator.

Section 13 - Pathology services—period for sharing information with the My Health Record system

Section 13 specifies the time period within which the information specified in section 12 must be uploaded to My Health Record. Information needs to be uploaded within 24 hours of when the information is first provided to any of the following:

- a. the healthcare provider who requested the healthcare to which the information relates;
- b. the healthcare provider who is treating the healthcare recipient to whom the information relates;
- c. the healthcare recipient to whom the information relates.

The 24 hour period supports technical implementation matters, such as where reports may be sent to My Health Record in batches, or scheduled for upload outside of normal business hours. This timeframe reflects expectations that information should be shared to My Health Record at the same time as it is shared to the healthcare recipient's requesting or treating healthcare provider, or to the recipient where the service was requested directly. This would include the requirement to upload partial results where a series of tests have been requested, but some finalised results are available and shared to a health care provider prior to all tests being completed.

Section 14 - Pathology services—kinds of health information that may be collected, used or disclosed

Section 14 specifies the types of health information contained in the My Health Record System that can be collected, used or disclosed for the purposes of enforcing the requirements of a share by default provision. For the purposes of section 70AA this information is limited to:

- a. the type of a report or other document (i.e. a pathology report - it will not specify what type of test or scan)
- b. the time at which, and the date on which, a pathology service is requested or rendered.

This information is able to be disclosed to the Chief Executive Medicare, the Secretary of the Department and any other Commonwealth entity specified in the Rules. This information is prescribed in the Instrument rather than the Act to ensure it can be limited to only what is necessary for compliance purposes, which may be different depending on the kinds of healthcare and information being specified. If different healthcare and information are specified for the purposes of section 78A(1) in the future, then different information may be required for compliance purposes under section 70AA, which would be subject to further consultation prior to prescribing the additional information.

Under subsection 70AA(4), healthcare recipient only notes are not able to be collected, used or disclosed for compliance purposes.

Statement of Compatibility with Human Rights

Prepared in accordance with Part 3 of the Human Rights (Parliamentary Scrutiny) Act 2011

My Health Record (Share by Default) Rules 2025

This Disallowable Legislative Instrument is compatible with the human rights and freedoms recognised or declared in the international instruments listed in section 3 of the *Human Rights (Parliamentary Scrutiny) Act 2011*.

Overview of the Disallowable Legislative Instrument

The *Health Legislation Amendment (Modernising My Health Record - Sharing by Default) Act 2025* (SBD Act) introduced reforms aimed at enhancing the accessibility and utility of health records in Australia for consumers and their care teams.

The SBD Act amended the *My Health Records Act 2012* (the Act) and *Health Insurance Act 1973* (HI Act) to implement a new regulatory framework to mandate the sharing of consumer health information to My Health Record. The Act was amended to create an obligation on specified constitutional corporations to become registered under the Act and to upload prescribed information. The HI Act was also amended to make medicare benefits for prescribed services only payable when prescribed information has been uploaded. The amendments in the SBD Act ensure that where an obligation applies to either a constitutional corporation or an individual healthcare provider or both, the obligation will be satisfied if either uploads the prescribed information.

The SBD Act is intended to support consistent and streamlined health information sharing while maintaining robust privacy protections. It embeds the principles of transparency, accountability, and consumer choice and control.

The initial scope of the proposed mandate is reports authorised by pathologists and radiologists as outlined in this Instrument as well as rules made under the HI Act. This Instrument specifies proprietors of pathology laboratories and diagnostic imaging premises as the constitutional corporations who will be required to become registered under the Act and upload prescribed information. The Instrument outlines the information required to be uploaded are pathologist and radiologist reports resulting from specified pathology and diagnostic imaging services, subject to limited exceptions. Uploading to My Health Record will be required within 24 hours of the report being sent to a requesting or treating practitioner. Reports will be required to be uploaded regardless of whether medicare is payable for those services.

Human rights implications

The Instrument engages the following rights:

- Article 12(1) of the International Covenant on Economic, Social and Cultural Rights (ICESCR) being the right to the enjoyment of the highest attainable standard of physical and mental health; and

- The right to privacy under Article 17 of the International Covenant on Civil and Political Rights (ICCPR).

The Right to Health

Article 12(1) of the ICESCR provides that everyone has the right to the enjoyment of the highest attainable standard of physical and mental health. To give effect to this right, better sharing of health information will support healthcare recipients and their healthcare providers to have access to information required to make informed decisions about their health and care planning. Better access to health information will also support greater accessibility of health information and services. Ensuring that healthcare recipients and their care team have timely access to their pathology and diagnostic imaging reports will reduce the need for unnecessary duplicate testing and speed up diagnosis and treatment times. It will also decrease the amount of time healthcare providers have to spend chasing up past reports.

The Strengthening Medicare Taskforce Report, released in 2022, highlighted that access to real time health information is a critical foundation for a modern and connected healthcare system. It recommended requiring that information be shared by default for private and public practitioners and services.

The sharing by default requirements in the Instrument will ensure consistent and timely health information sharing to support a healthcare recipient's healthcare journey across different healthcare settings.

The Instrument promotes the right to an adequate standard of living and the right to the enjoyment of the highest attainable standard of physical and mental health for Australians.

The Right to Privacy

The protection against arbitrary or unlawful interference with privacy is contained in Article 17 of the ICCPR. Article 17 provides that no one shall be subjected to arbitrary or unlawful interference with his or her privacy, family, home or correspondence, nor to unlawful attacks on their honour or reputation, and that everyone has the right to the protection of the law against such interference or attacks.

Although the United Nations Human Rights Committee has not defined 'privacy', it should be understood to comprise freedom from unwarranted and unreasonable intrusions into activities that society recognises as falling within the sphere of individual autonomy.

The right to privacy includes respect for informational privacy, including in respect of storing, using and sharing private information and the right to control the dissemination of personal and private information. The right to privacy also includes the right to the protection of one's personal data. The Human Rights Committee has said that, pursuant to Article 17(2) of the ICCPR, States are required to regulate the processing, use and conveyance of automated personal data, and to protect those affected against misuse. The Committee has said, moreover, that State Parties must take all appropriate measures to ensure that the gathering, storage and use of sensitive personal data is consistent with their obligations under Article 17.

The right to privacy under Article 17 can be permissibly limited in order to achieve a legitimate objective and where the limitations are lawful and not arbitrary. The term 'unlawful' in Article 17 of the ICCPR means that no interference can take place except as

authorised under domestic law. Additionally, the term ‘arbitrary’ in Article 17(1) of the ICCPR means that any interference with privacy must be in accordance with the provisions, aims and objectives of the ICCPR and should be reasonable in the particular circumstances. The Committee has interpreted ‘reasonableness’ to mean that any limitation must be proportionate and necessary in the circumstances.

To the extent that the share by default requirements in the Instrument authorise the collection, use or disclosure of personal or health information or may interfere with the right to privacy, such measures are lawful and nonarbitrary. The measure aims to achieve the legitimate objective of assuring the consistency and completeness of key health information as well as improved access to a healthcare recipient’s own information.

The limitations on the right to privacy under the Instrument are reasonable, necessary and proportionate as they appropriately balance the competing objectives of transparency and oversight with an individual’s right to privacy.

The SBD Act contains protections to ensure health information is being collected, used and disclosed in an appropriate and non-invasive manner to achieve legitimate public health aims and objectives.

The Act and HI Act both provide exceptions to the share by default requirements. Information cannot be uploaded to an individual’s My Health Record against their wishes as individual preference is a valid exception to the share by default requirement. Their nominated or authorised representative can also make this choice on behalf of the healthcare recipient if they are unable to make it themselves. An individual’s privacy is further safeguarded by a discretion that can be exercised by healthcare providers that sharing the individual’s health information would be detrimental to their health, safety or wellbeing.

The Instrument contains provisions that enable the sharing of health information for the purposes of managing compliance and enforcement with the share by default requirements. These requirements are necessary and proportionate to the objectives of the Instrument and SBD Act. Only very limited health information will be collected from the My Health Record system being:

- a. the type of a report or other document (i.e. pathology report or diagnostic imaging report - it will not specify what type of test or scan)
- b. the time at which, and the date on which, a service is requested or rendered.

This information will then be matched against Medicare claims information held by the Department to see if claims have been made for services for which there is no corresponding upload.

If there is no corresponding upload then the Department may request records of any applicable exceptions from a healthcare provider justifying not uploading the information. The contents of reports or any other consumer health information stored in the My Health Record system will not be collected, used or disclosed for any compliance purpose.

Conclusion

The Instrument is compatible with human rights because it promotes better health outcomes for Australians by enabling efficient and effective access to health services by trusted

providers. The Instrument engages the right to privacy for the legitimate objective of promoting better access to health services and to the extent that the right to privacy is limited, this is reasonable, necessary and proportionate in the circumstances.

Mark Butler
Minister for Health and Ageing